

Hernia - Female (Adult)

Algorithm 1418 | Adult (18 years and older) | Gender at birth: Female

Definition

- UAT assessment structure for a known or suspected abdominal-wall, groin, umbilical, or incisional hernia. It distinguishes a soft, uncomplicated bulge from incarceration, bowel obstruction, or strangulation, where trapped bowel or other tissue may lose its blood supply. A remote description cannot confirm the diagnosis or safely establish reducibility.
- Adult patient.
- Female.

Initial assessment

1. [OPEN_TEXT] What is the person's exact age and sex, and are they pregnant or recently postpartum? For an infant, was the baby premature?
2. [LOCATION] Where is the bulge or lump: groin, scrotum/labia, belly button, abdominal wall, or a previous-operation scar? Is this a clinician-diagnosed hernia?
3. [DURATION] When did it appear, and has its size, firmness, tenderness, colour, or usual ability to flatten when relaxed changed?
4. [YES_NO] Is there sudden, severe, persistent, or rapidly worsening pain; a hard/tender bulge; new redness, purple/dark colour, or marked swelling; or a bulge that no longer flattens as it normally does?
5. [YES_NO] Is there repeated or green vomit, inability to keep fluids down, a swollen or increasingly painful abdomen, inability to pass stool or gas, blood in vomit or stool, fever/shivering, confusion, faintness, or severe weakness?
6. [YES_NO] For a baby or child, are they inconsolable, unusually sleepy/floppy, feeding poorly, repeatedly vomiting, passing fewer wet nappies, or is there a painful/firm groin, scrotal, labial, or umbilical swelling?

Triage and redirect questions

1. Redirect → Nausea

Is nausea or vomiting the main concern without a new or known lump or bulge?

Nausea or vomiting without a hernia concern belongs in the Nausea pathway.

1. Disposition level 100

Is the hernia or bulge suddenly or severely painful, hard, tender, rapidly larger, newly red/purple/dark, or no longer flattening as it normally does? Is there repeated or green vomiting, inability to keep fluids down, a swollen/painful abdomen, inability to pass stool or gas, blood in vomit/stool, fever/shivering, confusion, faintness, or rapid deterioration? For a baby or child, include inconsolable crying, unusual sleepiness/floppiness, poor feeding, repeated vomiting, or a painful/firm groin, scrotal, labial, or umbilical lump.

A painful irreducible or discoloured hernia, especially with vomiting, distension, or failure to pass stool/gas, may represent incarceration, bowel obstruction, or strangulation requiring urgent surgery. Infants can deteriorate quickly and an irreducible inguinal hernia may also threaten bowel, testicular, or ovarian blood supply.

1. Disposition level 70

With all emergency features absent, is there new or increasing discomfort, tenderness, enlargement, uncertain reducibility, or a new groin/scrotal/labial lump; or is the patient an infant, pregnant/recently postpartum, or recently operated on?

A symptomatic, changing, uncertain, pediatric, pregnancy-associated, or postoperative lump needs prompt in-person examination. Remote assessment cannot confirm the type of hernia, exclude another diagnosis, or establish safe reducibility.

1. Disposition level 40

Is this an adult with a soft, painless, unchanged bulge that still flattens in its usual way when relaxed, with no vomiting, abdominal swelling, bowel change, fever, skin-colour change, pregnancy/postpartum concern, or other feature above?

An uncomplicated suspected hernia still requires an in-person clinical examination to confirm the diagnosis and discuss monitoring or repair. The Qatar route and timing require local governance approval.

Advice

Qatar emergency response for possible incarceration, obstruction, or strangulation

- Call Qatar emergency services on 999 now and follow the call-taker's instructions. Do not drive or allow the person to drive. Keep them resting in the position of greatest comfort and do not force, repeatedly manipulate, massage, bind, tape, or apply direct heat or ice to the bulge. Do not give food, drink, laxatives, enemas, or unprescribed medicine because urgent surgery or anaesthesia may be needed. If vomiting, place an alert person on their side or leaning safely forward; if unresponsive but breathing, use the recovery position. If they become unresponsive and are not breathing normally, start age-appropriate CPR as directed by 999. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
- Call Back If: severe or worsening pain; hard, tender, irreducible, enlarged, or discoloured bulge; repeated or green vomiting; abdominal distension or inability to pass stool/gas; infant poor feeding, floppiness, or inconsolability; confusion, faintness, fever, or rapid deterioration.

Prompt in-person assessment for a symptomatic or higher-risk hernia

- Arrange prompt same-day in-person assessment through the Qatar route approved by local clinical governance. Avoid heavy lifting, straining, or activity that increases pain. Do not ask the caller to test or force the lump back in, and do not bind or tape it. Follow only an existing clinician-taught plan for a known hernia. For an infant, pregnancy/postpartum patient, new groin/scrotal/labial lump, or postoperative lump, use the age- and service-appropriate route approved by governance. Call 999 immediately if pain becomes severe, the bulge becomes hard/tender/discoloured or stops flattening as usual, vomiting or abdominal swelling develops, stool/gas cannot pass, or the person's general condition worsens.
- Call Back If: increasing pain or swelling; hardness, tenderness, colour change, or loss of usual reducibility; vomiting, distension, or inability to pass stool/gas; baby becomes inconsolable, sleepy, floppy, or feeds poorly.

In-person confirmation of an uncomplicated suspected hernia

- Arrange a non-emergency in-person assessment through the Qatar route and timing approved by local governance; a photograph or video cannot confirm the diagnosis. Until reviewed, avoid activities that clearly provoke pain or bulging and address constipation or persistent cough with a clinician or pharmacist. Do not force reduction, bind or tape the bulge, begin strenuous exercise, or use this pathway for a child, pregnancy/postpartum concern, recent operation, pain, vomiting, abdominal swelling, bowel obstruction symptoms, fever, or colour change.
- Call Back If: new pain, firmness, tenderness, enlargement, or colour change; bulge no longer flattens as usual; vomiting, distension, constipation with inability to pass gas, fever, or feeling unwell.

First aid

- Call Qatar emergency services on 999 now and follow the call-taker's instructions. Do not drive or allow the person to drive. Keep them resting in the position of greatest comfort and do not force, repeatedly manipulate, massage, bind, tape, or apply direct heat or ice to the bulge. Do not give food, drink, laxatives, enemas, or unprescribed medicine because urgent surgery or anaesthesia may be needed. If vomiting, place an alert person on their side or leaning safely forward; if unresponsive but breathing, use the recovery position. If they become unresponsive and are not breathing normally, start age-appropriate CPR as directed by 999. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.

References

- Hamad Medical Corporation, "Life-Threatening Medical Emergency", <https://hamad.qa/EN/Emergency/Pages/Life-ThreateningMedicalEmergency.aspx> (accessed 2026-07-25)
- NHS.UK, "Hernia", <https://www.nhs.uk/conditions/hernia/> (page last reviewed 19 May 2026)
- NHS.UK, "Umbilical hernia", <https://www.nhs.uk/conditions/umbilical-hernia/> (accessed 2026-07-25)
- NHS England, "Making a decision about inguinal hernia", <https://www.england.nhs.uk/wp-content/uploads/2023/11/PRN00250-dst-making-a-decision-about-inguinal-hernia.pdf> (accessed 2026-07-25)
- Royal Children's Hospital Melbourne, "Irreducible inguinal hernia - pre-referral", https://www.rch.org.au/kidsconnect/prereferral_guidelines/Irreducible_inguinal_hernia_prereferral/ (accessed 2026-07-25)

Provenance

Shaped to mirror the licensed After-Hours Telehealth Triage Guidelines database structure. This is IST open-source guideline content derived from cited public sources; it is not licensed STCC content. Question.DispositionLevel=null marks a See More Appropriate Guideline redirect. Demographic variants contain only source-supported differences; identical thresholds are documented rather than fabricated. Local destinations and TelemedicineEligible values require IST clinical governance approval. UAT DATA ONLY - NOT FOR REAL-PATIENT CARE OR PRODUCTION USE.

Manifest hash: 818381a2d61515959fd880594436d99844ea28a290842ed3ed4db8ca71efc173 | Status: BLOCKED_BY_RESEARCH_GAP